Batterjee Medical College
For Science and Technology

Psychiatry · Clinical Lecture

Anxiety, OCD & Stress-Related Disorders

The anxiety spectrum, the obsessive-compulsive family, and the trauma- and stressor-related disorders.

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Learning Objectives (1 of 2)

Mapped to the BMC Knowledge / Skill / Value framework. Prerequisite: psychiatric history taking and the mental status examination.

Knowledge & understanding

- Define anxiety and say when it becomes a disorder
- Name the disorders in the three DSM-5 chapters
- State the diagnostic criteria and the duration thresholds

Skills

- Screen for anxiety, panic, OCD and trauma — in Arabic and English
- Ask WHY a situation is feared, to assign the diagnosis
- Separate panic disorder, agoraphobia and specific phobia

Values & attitudes

- Treat anxiety as a disabling illness, not a character trait
- Recognise the shame that hides taboo obsessions and hair-pulling
- Ask about suicide in OCD and BDD — the risk is under-rated

Learning Objectives (2 of 2)

Knowledge & understanding

- Distinguish an obsession from a compulsion
- Describe the course and prognosis of each disorder
- Outline first-line management across the chapter

Skills

- Distinguish acute stress disorder from PTSD by the clock
- Exclude substance, medication and medical causes first
- Explain exposure therapy and ERP to a patient

Values & attitudes

- Respect the cultural framing of distress and help-seeking
- Avoid long-term benzodiazepines even when the patient asks
- Aim for function, not zero anxiety

DSM-5 split the old "anxiety disorders" chapter into three — anxiety, obsessive-compulsive and related, and trauma- and stressor-related. This lecture follows that structure, and the split itself is examinable.

Fear vs anxiety — and what makes it a disorder

Fear

- An emotional response to a real or perceived IMMINENT threat
- Autonomic arousal, fight-or-flight
- Thoughts of immediate danger

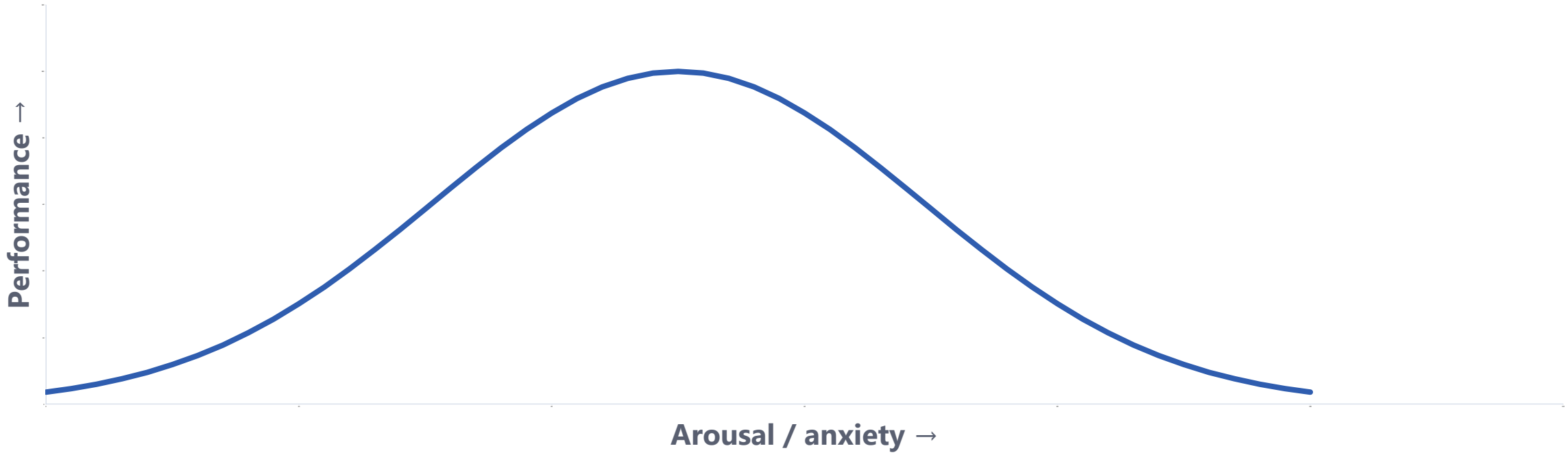
Anxiety

- Anticipation of a FUTURE threat
- Muscle tension and vigilance
- Disorders share excessive fear or worry, and avoidance
- They differ in WHAT is feared and the associated thoughts

Two universal rules for every diagnosis in this lecture: symptoms are not due to a substance, medication or medical condition, and they cause clinically significant distress or impairment.

The Yerkes–Dodson curve

A little arousal helps performance; too much impairs it. Anxiety becomes a disorder when it pushes you down the right-hand slope.



This is why "just relax" is bad advice and why zero anxiety is not the treatment goal — the aim is to move the patient back toward the peak, not to the left-hand end.

Signs & symptoms of anxiety — by system

System	Manifestations
Constitutional	Fatigue, diaphoresis (sweating), shivering
Cardiac	Chest pain, palpitations, tachycardia, hypertension
Pulmonary	Shortness of breath, hyperventilation
Neuro / musculoskeletal	Vertigo, light-headedness, paraesthesias, tremor, insomnia, muscle tension
Gastrointestinal	Abdominal discomfort, anorexia, nausea, vomiting, diarrhoea, constipation

Anxiety is a whole-body experience — patients often present to cardiology, the ED or GI clinics long before they reach psychiatry.

Organic & drug causes of anxiety

ALWAYS EXCLUDE FIRST

Medications & substances

- Caffeine; stimulants — cocaine, amphetamines
- Sympathomimetics (decongestants)
- Bronchodilators (theophylline, salbutamol), thyroxine, steroids
- WITHDRAWAL: alcohol, benzodiazepines, opioids, nicotine

Medical conditions

- Hyperthyroidism, phaeochromocytoma, hypoglycaemia
- Cardiac / pulmonary: arrhythmia, MI, PE, asthma, COPD
- Neurological: seizure, vestibular disease

New-onset anxiety in an older patient with no history — screen for a medical cause, especially thyroid, cardiac and caffeine, before labelling it a primary anxiety disorder.

The anxiety disorders

Childhood-onset

- Separation anxiety disorder
- Selective mutism
- Specific phobia

The common adult ones

- Social anxiety disorder (social phobia)
- Panic disorder
- Agoraphobia

The rest

- Generalised anxiety disorder
- Substance- or medication-induced
- Due to another medical condition

A key DSM-5 change: agoraphobia and panic disorder are now SEPARATE, independent diagnoses. Panic attacks can be a specifier attached to any disorder.

General screening approach

Ask these first

- Core screen — "Do you worry a lot? Do you get sudden rushes of fear? Do you avoid places or situations? Do you have repetitive thoughts or rituals?"
- Panic screen — "A sudden surge of fear peaking within minutes, with palpitations, chest tightness, choking and a fear of dying?"

Then localise

- WHAT is feared or avoided, and WHY — the feared consequence separates the disorders
- Duration and functional impairment
- Rule out a substance or medical cause, and depression

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Overview of management

Psychotherapy

- CBT is first-line across most disorders
- Exposure-based therapy for phobias, panic and agoraphobia
- ERP for OCD

Pharmacotherapy

- SSRIs and SNRIs are first-line
- Benzodiazepines short-term only, then tapered
- Drug detail lives in the Psychopharmacology lecture

General

- Psychoeducation; reduce caffeine
- Treat comorbid depression and substance use
- Continue treatment to prevent relapse

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PART 1

The Anxiety Disorders

GAD · panic disorder · agoraphobia · social anxiety · specific phobia · separation anxiety.

How long must the worry last?

A 34-year-old woman describes uncontrollable worry about work, her children and her health, with restlessness, poor sleep and muscle tension.

She wonders if "it's just stress".

Minimum duration for GAD?

A 2 weeks

B 1 month

C 3 months

D 6 months, more days than not

Commit to an answer before the next slide.

How long must the worry last?

ANSWER

A 34-year-old woman describes uncontrollable worry about work, her children and her health, with restlessness, poor sleep and muscle tension.

She wonders if "it's just stress".

Minimum duration for GAD?

A 2 weeks

B 1 month

C 3 months

D **6 months, more days than not**

D — ≥ 6 months, more days than not.

GAD requires excessive, hard-to-control worry for at least 6 months plus ≥ 3 of 6 physical symptoms. The 6-month threshold recurs across most anxiety disorders.

GAD — epidemiology

Lifetime

- ~6% of adults
- 3% of children
- 11% of adolescents

Onset

- Mean age ~33
- Often presents in the 20s

Sex

- About twice as common in women

Comorbidity

- Elevated risk of medical conditions
- Pure GAD is the exception, not the rule

GAD is the anxiety disorder that most often coexists with another mental disorder.

GAD — the criteria

WATCHERS

Excessive anxiety and worry, more days than not, for ≥ 6 months, about several events or activities — and the worry is difficult to control. Plus ≥ 3 of 6 symptoms in adults (only 1 is needed in children).

W Worry

Excessive and hard to control

A Anxiety

About several events or activities

T Tension

Muscle tension

C Concentration

Difficulty concentrating; mind going blank

H Hyperarousal

Irritability

E Energy loss

Easily fatigued

R Restlessness

Feeling on edge

S Sleep

Sleep disturbance

The six countable symptoms are restlessness, fatigue, poor concentration, irritability, muscle tension and sleep disturbance — you need three.

GAD — exclusions & the MDD overlap

Not better attributed to

- A substance or medical condition — e.g. hyperthyroidism
- Panic disorder, social anxiety, OCD, separation anxiety

GAD vs MDD

- Up to 80% of depressed patients report anxiety
- Overlapping symptoms: fatigue, sleep, concentration
- To diagnose GAD, its symptoms must be present IN THE ABSENCE of MDD

Anxiety occurring only during a depressive episode is a symptom of depression, not GAD. Marked distress or dysfunction is required either way.

GAD — course

Presentation

- Age of onset is difficult to specify
- Usually reaches a clinician in the 20s
- Only about ⅓ of patients seek psychiatric treatment

Course

- Chronic — may be lifelong
- Severity fluctuates over time
- Often worse with stress

The panic attack — 4 of 13 symptoms

An abrupt surge of intense fear or discomfort peaking within minutes, from a calm or an anxious state. A panic attack is a building block, not a diagnosis.

Cardiorespiratory

- Palpitations
- Shortness of breath
- Chest pain
- Choking feeling

Autonomic & neuro

- Sweating
- Dizziness
- Trembling or shaking
- Paraesthesias
- Chills or heat sensations
- Nausea or abdominal distress

Cognitive

- Fear of dying
- Fear of "going crazy" or losing control
- Derealisation or depersonalisation

Panic disorder

Core

- Recurrent, UNEXPECTED panic attacks
- ≥ 1 attack followed by ≥ 1 month of one or both of:
- – Anticipatory anxiety — worry about more attacks
- – A maladaptive change in behaviour — avoidance

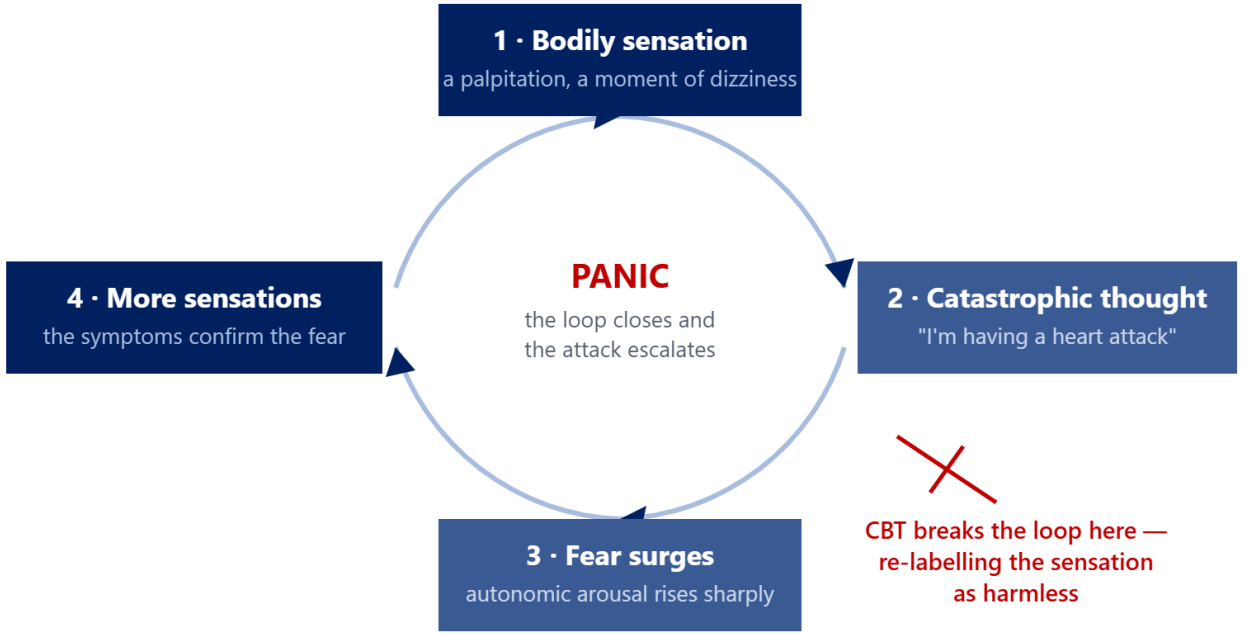
Not better explained by

- A substance or medical condition — cardiac disease, hyperthyroidism
- Another disorder — social anxiety, specific phobia, OCD, PTSD, separation anxiety

The "unexpected, out-of-the-blue" quality is what distinguishes panic disorder from the cued attacks seen in phobias.

The panic cycle

Catastrophic misinterpretation turns a harmless bodily sensation into a panic attack.



CBT breaks the loop by re-labelling the bodily sensations as harmless — interrupting the catastrophic interpretation rather than trying to suppress the sensation.

Panic disorder — course & treatment

Course

- Onset in late adolescence or early adulthood
- Generally chronic
- 91% have another psychiatric disorder
- Two-thirds occur during or after MDD

Prognosis

- 30–40% symptom-free long-term
- ~50% mild residual symptoms
- 10–20% significant ongoing symptoms

Management

- SSRI/SNRI plus CBT
- Benzodiazepine short-term only (4–12 weeks), tapered slowly
- β-BLOCKERS ARE NOT USEFUL

Agoraphobia — ≥ 2 of 5 situations

The five situations

- Public transportation
- Open spaces
- Enclosed places
- Standing in line or in a crowd
- Being outside the home alone

The rules

- Marked fear about ≥ 2 of the five
- The situations almost always provoke fear
- Actively avoided, or need a companion, or endured with intense anxiety
- ≥ 6 months; significant distress or dysfunction

Agoraphobia — why the avoidance?

The feared consequence

- Thoughts that escape might be difficult, or help unavailable, if panic-like or incapacitating or embarrassing symptoms occur
- e.g. fear of falling in the elderly, fear of incontinence

Course & management

- Onset around 20; later onset is not uncommon
- Chronic and unremitting without treatment
- Agoraphobia WITHOUT panic is the most persistent form
- Treatment as for panic disorder — SSRI/SNRI plus exposure-based CBT

It is the feared CONSEQUENCE — not the situation — that makes this agoraphobia rather than a specific phobia.

Social anxiety disorder (social phobia)

Criteria

- Marked fear of ≥ 1 social situation with possible scrutiny — speaking, meeting people, eating in public
- Fear of being negatively evaluated or humiliated
- Situations avoided or endured with intense fear; out of proportion
- ≥ 6 months; marked distress or dysfunction

Notes

- If a medical condition is present — Parkinson's, obesity, disfigurement — the fear is unrelated to it or clearly excessive
- Specifier: PERFORMANCE-ONLY type

Social anxiety — course & treatment

% Epidemiology

- Lifetime 3–13%
- Onset in the teens, range 5–35

Prognosis

- Half to two-thirds respond in trials
- About half of responders fully remit
- Continue treatment 6–12 months
- CBT benefits persist up to 5 years

The pearl

- Generalised type — SSRI/SNRI plus CBT
- PERFORMANCE-ONLY type — propranolol or a benzodiazepine PRN before the event

Contrast with panic disorder, where β-blockers are not useful — the distinction is commonly examined.

Specific phobia

Criteria

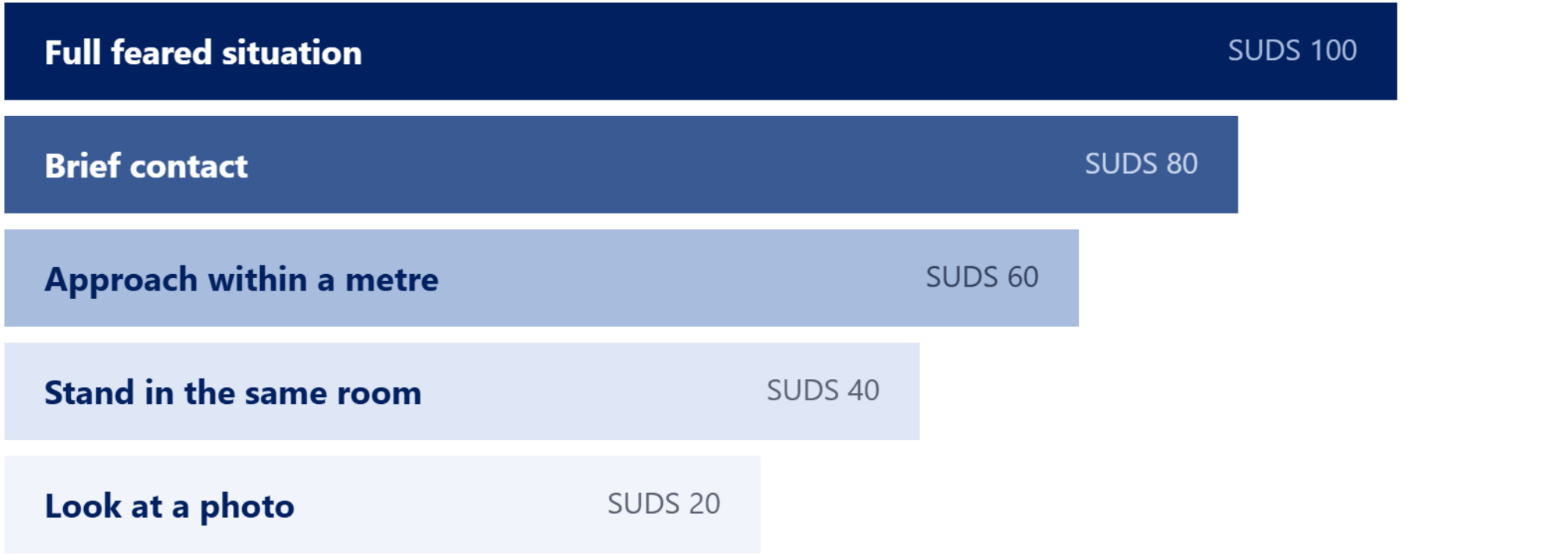
- Marked fear of a specific object or situation, which almost always provokes immediate fear
- Actively avoided or endured with intense anxiety
- Out of proportion to the actual danger
- ≥ 6 months; distress or dysfunction

Epidemiology

- Lifetime 10–13%; women > men
- Onset 5–12 years
- Animal and blood-injection-injury types have earlier onset than situational or natural-environment types

Blood-injection-injury phobia is unique — it can cause a VASOVAGAL FAINT (bradycardia and hypotension) rather than tachycardia, and is managed with applied tension.

The exposure hierarchy ("fear ladder")



Stay — remain in each step until anxiety falls (habituation)

Then climb — only move up once it has

Never escape at peak — that is what maintains the phobia

The mechanism is habituation — which is why exposure is graded and agreed with the patient rather than confrontational.

Specific phobia vs agoraphobia

The distinction

- Agoraphobia: ≥ 2 situations, avoided because of panic-like or embarrassing symptoms
- Specific phobia: ONE situation, and the fear is not related to panic-like symptoms

Worked cases

- Fear of airplanes and trains only → specific phobia
- Fear of flying because the plane will crash → specific phobia
- Fear of airplanes, elevators AND crowds → agoraphobia

Ask WHY the situation is feared — the feared consequence, not the situation itself, assigns the diagnosis.

Separation anxiety disorder — ≥ 3 features

Excessive fear of separation from an attachment figure.

Features

- Distress on separation from home or attachment figures
- Worry about losing them, or harm befalling them
- Worry that an untoward event — getting lost, kidnapped — will cause separation

...continued

- Refusal to go out or away from home
- Fear of being alone, or of sleeping away from home
- Duration: > 4 weeks in children, > 6 months in adults
- Not better explained by autism, psychosis, agoraphobia or GAD

Separation anxiety — across the lifespan

Then & now

- Grouped with childhood disorders from DSM-III through DSM-IV-TR
- DSM-5 recognises that it occurs in ADULTS

Epidemiology

- Lifetime: 4.1% in childhood, 6.6% in adults
- Most adults had first onset IN ADULTHOOD — only about a third are childhood-onset
- Attachment figure: children → parents; adults → spouse or friend

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PART 2

OCD & Related Disorders

OCD · body dysmorphic disorder · hoarding · trichotillomania · excoriation.

Obsessive-compulsive & related disorders

The unifying theme: repetitive thoughts and behaviours.

OCD

Obsessions, compulsions, or both.

Body dysmorphic disorder

Preoccupation with a perceived appearance flaw.

Hoarding disorder

Difficulty discarding possessions, with clutter.

Trichotillomania

Recurrent hair-pulling causing hair loss.

Excoriation

Recurrent skin-picking causing lesions.

Secondary

Substance-induced, or due to a medical condition.

Obsession or compulsion?

A patient is tormented by intrusive, unwanted images that he might have left the stove on.

To relieve the dread, he returns to check it exactly seven times before he can leave.

The checking behaviour is a…

A Obsession

B Compulsion

C Delusion

D Tic

Commit to an answer before the next slide.

Obsession or compulsion?

ANSWER

A patient is tormented by intrusive, unwanted images that he might have left the stove on.

To relieve the dread, he returns to check it exactly seven times before he can leave.

The checking behaviour is a…

A Obsession

B **Compulsion**

C Delusion

D Tic

B — a compulsion.

The intrusive image is the obsession (thought); the repetitive checking done to neutralise distress is the compulsion (behaviour or mental act). Obsessions raise anxiety; compulsions temporarily relieve it.

OCD — obsessions & compulsions

Obsessions

- Recurrent, persistent thoughts, urges or images
- Intrusive and UNWANTED
- Attempts to ignore, suppress or neutralise them — with another thought or an action

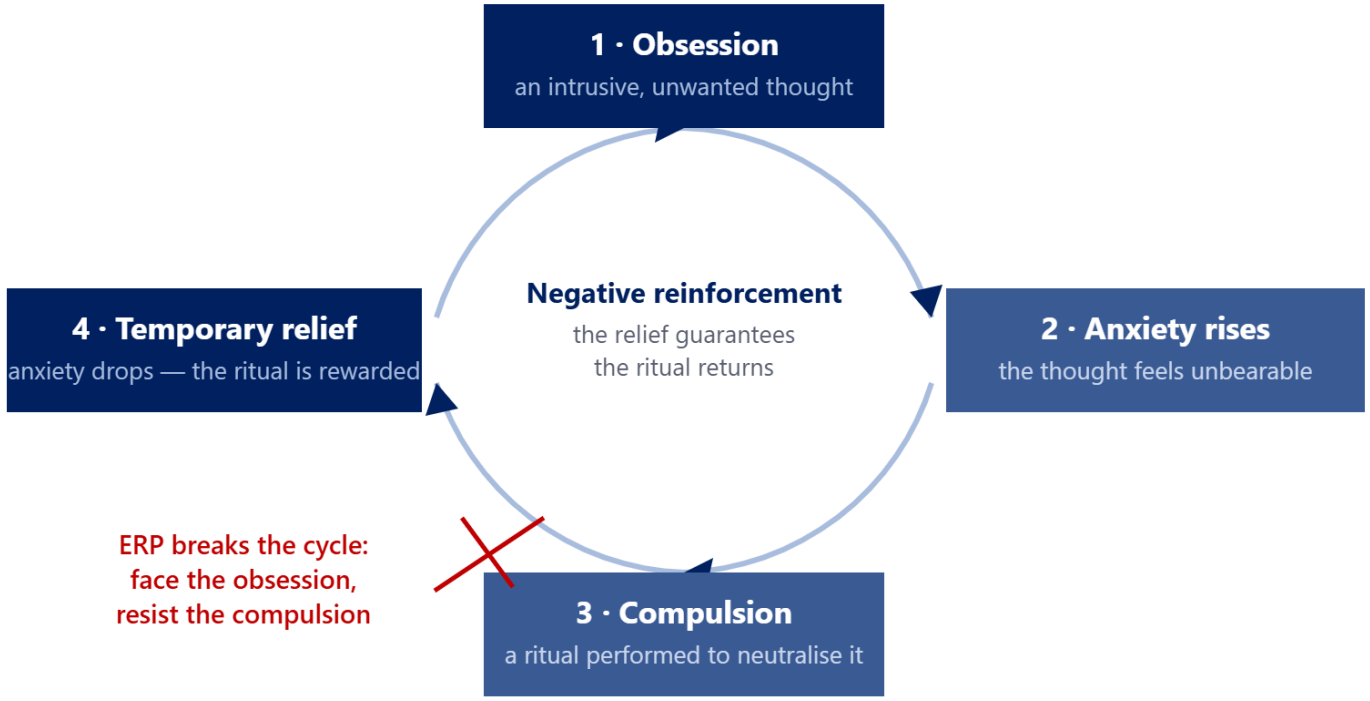
Compulsions

- Repetitive behaviours or MENTAL ACTS the person feels driven to perform in response to an obsession or rigid rules
- Aimed at preventing or reducing distress
- But not realistically connected to it, or clearly excessive

Presence of obsessions, compulsions, or both. The compulsion is a RESPONSE to the obsession — that link is the core of the disorder.

The OCD vicious cycle

The compulsion works — briefly. That is precisely the problem.



ERP breaks the cycle: face the obsession (exposure) and resist the compulsion (response prevention). Anxiety then falls on its own — habituation — and the reinforcement is broken.

Common obsessions → their compulsions

Obsession theme	Typical compulsion	Clinical clue
Contamination	Washing, cleaning	Chapped hands; avoids handshakes and doctors
Doubt (harm, safety)	Checking locks, stove, gas	Late leaving home; re-checks repeatedly
Symmetry / "just right"	Ordering, arranging, counting	Tasks never feel finished
Forbidden / taboo thoughts	Mental rituals, reassurance-seeking	Hidden; high shame; high suicide risk
Fear of losing things	Saving — now a separate hoarding diagnosis	Distinguish from hoarding disorder

Compulsions can be behavioural (washing, checking) or purely MENTAL (counting, praying, reviewing) — ask about both, or you will miss "pure-O" presentations.

OCD — the rest of the criteria

Threshold: time-consuming — more than 1 hour a day — OR marked distress or dysfunction, and not attributable to a substance, medical or other psychiatric disorder.

Insight specifier	The patient believes…	Why it matters
Good or fair	The obsessional beliefs are probably NOT true	The usual presentation; best prognosis
Poor	The beliefs are probably true	Worse response to treatment
Absent / delusional	Completely convinced the beliefs are true	Worst prognosis — and easily MISTAKEN FOR PSYCHOSIS

Poor or absent insight predicts a worse prognosis — and delusional-level OCD is treated as OCD, not as schizophrenia.

OCD — course & prognostic factors

Course

- Mean onset ~19.5 years; usually gradual
- Untreated: chronic, waxing and waning
- Suicidal ideation in ~50%, attempts in ~25%

Good prognostic factors

- Non-severe symptoms
- No comorbid obsessive-compulsive personality disorder
- Depressed or anxious mood rather than schizotypal
- Compliance with treatment; family support

The suicide figures are the ones students under-rate — half of OCD patients have suicidal ideation and a quarter attempt.

OCD & quality of life

Relationships

- Harm obsessions make relationships feel hazardous
- The patient withdraws

Work & study

- Symmetry and "just right" obsessions mean projects never finish
- School failure and job loss follow

Health care

- Contamination fears make the patient avoid physicians
- Excessive washing causes dermatological damage

Its own treatment

- Symptoms obstruct the cure
- Medications may be seen as "contaminated" and refused

OCD — treatment

First-line

- CBT with exposure and response prevention (ERP), medication, or both
- SSRI at MAXIMUM dose for ≥ 6 weeks
- OCD often needs HIGHER doses than depression does

If inadequate

- Switch to another SSRI, clomipramine, or venlafaxine
- Augment with an antipsychotic — e.g. risperidone 0.5–3 mg/day
- SSRIs and clomipramine improve 40–60% of patients

ERP is the single most effective intervention for OCD — medication is an alternative or adjunct when ERP is unavailable or declined.

Body dysmorphic disorder

Core

- Preoccupation with ≥ 1 perceived defect or flaw in appearance, not observable or only slight to others
- Repetitive behaviours — mirror-checking, grooming, skin-picking, reassurance-seeking — or mental acts such as comparing appearance

Boundaries

- Marked distress or dysfunction
- Not better explained by an eating disorder, where the concern is with weight or fat

A MUSCLE DYSMORPHIA specifier exists — the preoccupation that one's body is too small or insufficiently muscular — and is often missed in men.

Ms. R, 28 — "they laugh at how ugly I am"

Ms. R believes she is "ugly" and that others laugh at her. In reality she is attractive.

It began at 13, when she became obsessed with "facial defects" — her nose too fat, her eyes too far apart. She withdrew socially, her concentration fell and her grades dropped.

She picks at "blemishes", checks mirrors and reflective surfaces repeatedly, and thinks about the defects all day.

Course: lifetime prevalence ~2%, slightly higher in females. Onset is usually in adolescence, gradual or abrupt, with a long undulating course and few symptom-free intervals. Suicide attempts run at 6–23 times the rate of the general population.

BDD — management

Engage first

- Educate about the illness and the treatments
- Focus on the patient's SUFFERING and impaired functioning
- Do NOT comment on their looks — reassurance about appearance does not help and can entrench the preoccupation

Treat

- Mild to moderate: SSRI or CBT
- Non-responders: clomipramine
- Maintain treatment 3–4 years

Patients often seek cosmetic or dermatological procedures instead of psychiatric care — these rarely help and may worsen the preoccupation.

Hoarding disorder

Core

- Persistent difficulty discarding possessions, REGARDLESS OF VALUE
- Due to a perceived need to save, and distress on discarding
- Results in clutter that congests living areas — uncluttered only via third parties

Boundaries

- Marked distress or dysfunction
- Not due to another medical condition — brain injury, CVA, Prader-Willi
- Not better explained by another mental disorder

Specify with excessive acquisition (present in most cases) and the level of insight.

Hoarding — course & management

	Detail
Prevalence	~1.5% (England); equal in men and women
Trajectory	Emerges age 11–15 → interferes by the mid-20s → significant impairment by the mid-30s
Course	Chronic, waxing and waning
FIRST priority	Address SAFETY RISKS — fire, falls. Forcible action if children or elderly people are at risk
Treatment	CBT (limited availability) or an SSRI — the SSRI especially where there is comorbid mood or anxiety disorder

Hoarding is the one disorder in this lecture where safety may force action before any therapeutic alliance exists.

Trichotillomania (hair-pulling disorder)

Criteria

- Recurrent pulling out of one's hair, resulting in hair loss
- Repeated attempts to decrease or stop
- Marked distress or dysfunction
- Not due to a dermatological condition or another mental disorder (e.g. BDD)

Case — Ms. C, 27

- Started at 11, at the nape of the neck
- Progressed to the whole scalp with bald patches hidden by scarves
- Otherwise high-functioning — near completion of a master's
- Pulling is daily and often automatic; stopping causes rising tension

Trichotillomania — course & management

Two patterns

- Early to mid adolescence — F:M 10:1, serious and chronic
- Childhood onset — F = M, less serious, more common

Complication

- 5–40% chew or swallow the hair
- About a third of those develop BEZOARS (hairballs)
- Potentially hazardous, and sometimes surgical

Treatment

- No consensus
- SSRI, or CBT with habit reversal
- Augment with an antipsychotic if that fails

Ask every hair-pulling patient whether they swallow the hair — it changes the management entirely.

Excoriation (skin-picking) disorder

Criteria

- Recurrent skin picking, resulting in skin lesions
- Repeated attempts to decrease or stop
- Marked distress or dysfunction
- Not due to a substance (e.g. cocaine) or another medical or mental disorder

Clinical

- Often targets the face, arms and hands; picks at real or perceived blemishes
- Onset often at puberty; female predominance
- Comorbid with OCD, BDD and trichotillomania
- Treatment: CBT with habit reversal ± SSRI

Trichotillomania and excoriation are the BODY-FOCUSED REPETITIVE BEHAVIOURS — the pull or pick is often soothing, not distress-driven like a classic compulsion.

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PAUSE

Break — 10 minutes

After the break: the trauma- and stressor-related disorders — acute stress disorder, PTSD and adjustment disorders — then the synthesis tables, the case and the board questions.

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PART 3

Trauma- & Stressor-Related

Acute stress disorder · PTSD · adjustment disorders.

Same trauma, which diagnosis?

Ten days after a car crash, a man has nightmares, flashbacks, hypervigilance and emotional numbing.

Symptoms began the day after the crash.

Most likely diagnosis now?

A PTSD

B Acute stress disorder

C Adjustment disorder

D GAD

Commit to an answer before the next slide.

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Symptoms began the day after the crash.

Most likely diagnosis now?

A PTSD

B **Acute stress disorder**

C Adjustment disorder

D GAD

B — acute stress disorder.

Symptoms lasting 3 days to 1 month after trauma = ASD. If they persist beyond 1 month, the diagnosis becomes PTSD. Same symptom clusters, different timeline.

Acute stress disorder vs PTSD



Same four symptom clusters — re-experiencing · avoidance · mood and cognition · arousal. **Only the clock differs.**

Both require exposure to a traumatic event and significant impairment of function — the criteria that sit beyond the clock.

PTSD / ASD — core symptoms

1 · Re-experiencing

- Flashbacks and nightmares
- Intrusive memories
- Cues cause distress

2 · Avoidance

- External — places, people, conversations
- Internal — emotions and memories

3 · Mood & cognition

- Amnesia for part of the event
- Negative beliefs; self- or other-blame
- Persistent negative emotions
- Detachment and anhedonia

4 · Arousal

- Hypervigilance, exaggerated startle
- Irritability and anger
- Poor sleep and concentration

The same four clusters apply to both acute stress disorder and PTSD — only the duration differs.

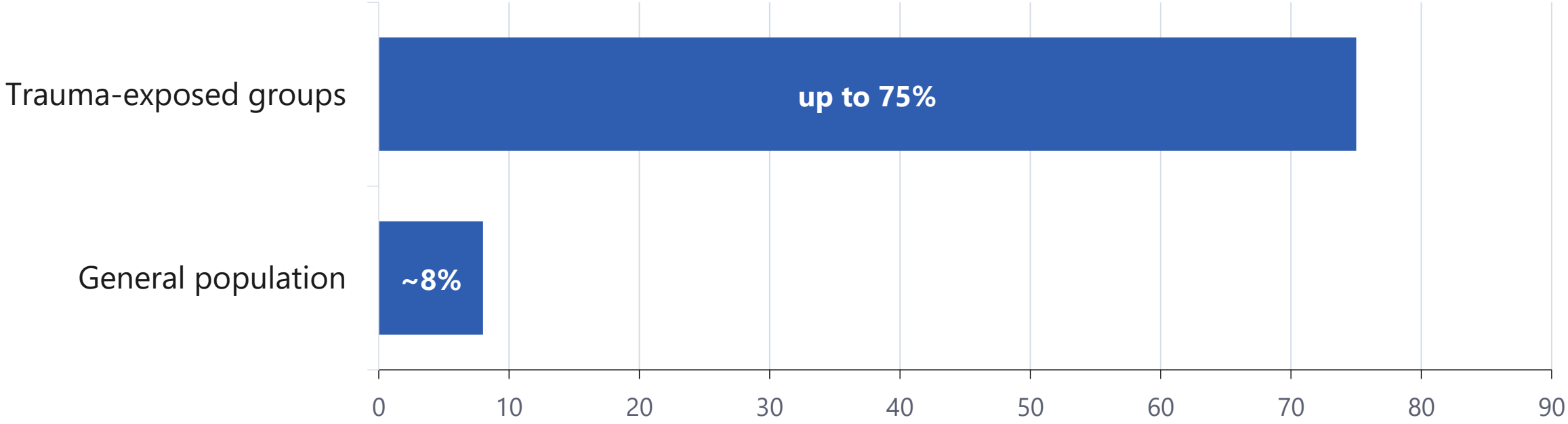
PTSD — who & how common

~8%
lifetime, general population

up to 75%
in high-risk trauma-exposed groups

5–15%
subclinical forms

Lifetime prevalence of PTSD (%) — exposure is what drives the difference



Risk factors: MOST IMPORTANT is the severity, duration and proximity of the trauma exposure. Also single, divorced or widowed; socially withdrawn; low socioeconomic level. The dose of trauma matters more than any personal characteristic.

PTSD — comorbidity & outcome

Comorbidity — high

- Two-thirds have ≥ 2 other disorders
- Depression, substance use, other anxiety disorders, bipolar disorder

Prognosis untreated

- ~30% recover completely; 40% mild; 20% moderate; 10% unchanged or worse
- About 50% recover by one year
- Good prognosis: rapid onset, short duration (< 6 months), good premorbid function, strong support, no other disorders

PTSD / ASD — management

Psychotherapy — first-line

- TRAUMA-FOCUSED CBT — exposure therapy, cognitive processing therapy
- EMDR
- Supportive and psychodynamic therapy; couples and family therapy

Pharmacotherapy

- First-line: SSRIs (sertraline, paroxetine) or SNRIs (venlafaxine)
- PRAZOSIN (α1-antagonist) for nightmares and hypervigilance
- Augment with a second-generation antipsychotic in severe or resistant cases

BENZODIAZEPINES ARE GENERALLY AVOIDED in PTSD — they do not treat the core symptoms and carry dependence and symptom-worsening risk.

Adjustment disorder

Definition

- An emotional or behavioural response to an identifiable stressor — financial, medical illness, relationship
- Onset WITHIN 3 MONTHS of the stressor
- Remits within 6 months of the stressor's removal
- Out of proportion, or significant impairment — not just normal bereavement

Subtypes, by predominant symptom

- With depressed mood
- With anxiety
- Mixed anxiety and depressed mood
- With disturbance of conduct
- Mixed emotions and conduct · unspecified

Adjustment disorder — management

Psychotherapy — the mainstay

- Supportive psychotherapy
- CBT
- Group therapy

Problem-solving

- Work on the STRESSOR itself
- Often the single most useful intervention — and the one most often skipped

Medication

- Only if clinically indicated
- Targets specific symptoms — insomnia, anxiety or depression
- Time-limited

Adjustment disorder is a DIAGNOSIS OF EXCLUSION — used when the reaction is clinically significant but does not meet criteria for a specific disorder such as MDD or PTSD.

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PART 4

Synthesis & Review

Neurobiology pearls · management-at-a-glance · the duration cheat-sheet · case · board questions · take-home.

Neurobiology & classic associations

Circuits & chemistry

- AMYGDALA — the fear/threat centre, over-active in anxiety and PTSD
- OCD: the cortico-striatal-thalamic loop — caudate and basal ganglia; serotonin plus glutamate
- Neurotransmitters: reduced GABA and serotonin, raised noradrenaline (locus coeruleus)

Tested triggers & links

- Panic provoked by sodium lactate infusion, CO_2 inhalation, caffeine
- PANDAS: paediatric OCD and tics after group-A strep (autoimmune)
- OCD associated with Tourette's and Sydenham chorea
- PTSD: small hippocampus, low cortisol, high CRF

First-line treatment across the disorders

Disorder	First-line	Pearls
GAD	SSRI/SNRI + CBT	Buspirone adjunct; benzodiazepine short-term only
Panic / agoraphobia	SSRI/SNRI + CBT (exposure)	β-blockers NOT useful; benzodiazepine bridge, then taper
Social anxiety	SSRI/SNRI + CBT	Performance-only → propranolol PRN
Specific phobia	Exposure therapy	Medication has a limited role
OCD / BDD	ERP + SSRI at high dose	Clomipramine; augment with an antipsychotic
PTSD	Trauma-focused CBT; SSRI/SNRI	Prazosin for nightmares; AVOID benzos
Adjustment	Supportive psychotherapy	Medication only for target symptoms

The backbone is the same everywhere — SSRI/SNRI plus a behavioural therapy. What changes is which therapy, and the exceptions in the right-hand column.

Every disorder at a glance

Disorder	Key feature	Duration	Onset	Prevalence	Sex
GAD	Uncontrollable worry + ≥ 3 physical Sx	≥ 6 mo	~33	~6%	F 2:1
Panic disorder	Recurrent unexpected attacks	≥ 1 mo worry	late teens–20s	~2–3%	F > M
Agoraphobia	≥ 2 situations, fear no escape	≥ 6 mo	~20	~1–2%	F > M
Social anxiety	Fear of scrutiny	≥ 6 mo	teens	3–13%	F > M
Specific phobia	One object or situation	≥ 6 mo	5–12	10–13%	F > M
Separation anxiety	Fear of losing attachment figure	> 4 wk / > 6 mo	child & adult	4–7%	F > M
OCD	Obsessions + compulsions	> 1 hr/day	~19.5	~2–3%	M = F
PTSD	Trauma + 4 symptom clusters	> 1 mo	any	~8%	F > M

Note the two that differ from the pattern: OCD is the only one with equal sex distribution, and it is graded by TIME SPENT rather than months of duration.

"How long?" — the time thresholds

3 days – 1 month · **≥ 1 month** · **≥ 6 months**

minimum duration required →

The trauma window

- ACUTE STRESS DISORDER — 3 days to 1 month after the trauma
- Under 3 days there is no diagnosis: acute distress is normal
- Same four symptom clusters as PTSD

The one-month group

- PTSD — beyond 1 month after the trauma
- PANIC DISORDER — ≥ 1 attack plus ≥ 1 month of worry or avoidance
- SEPARATION ANXIETY in children — > 4 weeks

The default

- GAD — more days than not
- SOCIAL ANXIETY · SPECIFIC PHOBIA · AGORAPHOBIA
- SEPARATION ANXIETY in adults
- Six months is the rule; the trauma disorders are the exception

Two sit off the axis. OCD is graded by TIME SPENT — more than an hour a day — not by months. ADJUSTMENT DISORDER is defined by its relation to the stressor: onset within 3 months, remitting within 6 months of the stressor going.

Mrs. A, 29 — one year of "heart attacks"

Mrs. A is a 29-year-old teacher referred by cardiology after three normal ECGs, troponins and an echocardiogram.

Over the past year she has had sudden episodes of pounding heartbeat, chest tightness, breathlessness, dizziness and tingling in her hands. Each peaks within about five minutes and passes in half an hour. The first came out of the blue while she was watching television.

She now worries daily that another one will come, and has stopped driving on the ring road, using lifts and going to the mall alone — she will only go if her sister comes with her.

She drinks five cups of Arabic coffee a day and sleeps poorly. She has lost weight, but her TSH is normal. She denies low mood most of the day, and there is no trauma history.

Case problem — work through it

Questions

- 1 · What is the most likely diagnosis — and is there a second one?
- 2 · Which criteria does she meet, and for how long must they have lasted?
- 3 · What organic causes must you exclude, and what in the history points to them?
- 4 · What is your first-line management, pharmacological and non-pharmacological?

Model answer

- 1 · Panic disorder — recurrent unexpected attacks plus ≥ 1 month of anticipatory anxiety and avoidance — WITH agoraphobia as a separate diagnosis: ≥ 2 of the five situations, needing a companion, ≥ 6 months
- 2 · Attacks peak within minutes with ≥ 4 of 13 symptoms; the avoidance is driven by fear that escape would be difficult or help unavailable
- 3 · Hyperthyroidism (TSH normal here), arrhythmia or MI (cardiac work-up negative), and caffeine — 5 cups a day is a genuine contributor to reduce
- 4 · SSRI/SNRI plus CBT with exposure; benzodiazepine only as a short-term bridge, tapered; β-blockers are NOT useful; cut caffeine; treat the sleep

Finish by asking what would change if the attacks only ever happened in lifts — the answer being a specific phobia, not panic disorder.

Board-style questions

#	Question	Answer
1	Panic-like symptoms 3 weeks post-trauma — ASD or PTSD?	Acute stress disorder (< 1 month). PTSD needs > 1 month
2	Most effective therapy for OCD?	Exposure and response prevention (ERP); SSRIs often at higher-than-antidepressant doses
3	Drug for performance-only social anxiety before a presentation?	Propranolol PRN — note β-blockers are NOT useful in panic disorder
4	Agoraphobia requires fear of how many situations, and why avoided?	≥ 2 of 5; avoided because escape or help would be hard if panic-like symptoms occur
5	PTSD drug specifically targeting nightmares?	Prazosin (α1-antagonist)
6	Paediatric OCD and tics after streptococcal infection?	PANDAS

Summary — seven things to remember

Diagnosis

- Always exclude a medical or substance cause first
- The FEARED CONSEQUENCE — not the situation — separates the disorders
- Most anxiety disorders need ≥ 6 months; ASD < 1 month, PTSD > 1 month
- Obsession = the intrusive thought; compulsion = the neutralising act

Treatment

- SSRI/SNRI plus CBT is the backbone across the board
- OCD needs ERP and often higher SSRI doses
- PTSD: trauma-focused CBT + SSRI; prazosin for nightmares; avoid benzodiazepines

References

Classification framed as DSM-5-TR throughout.

Course official textbooks

- DSM-5-TR — Diagnostic and Statistical Manual of Mental Disorders, 5th ed., Text Revision (APA, 2022)
- Kaplan & Sadock's Synopsis of Psychiatry

Others

- Oxford Handbook of Psychiatry
- UpToDate — anxiety, OCD and trauma-related topic reviews

Pharmacology is kept high-level here by design — the deep drug detail lives in the Psychopharmacology lecture. Image and icon credits: _assets\images\ATTRIBUTIONS.md.

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Thank You

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Questions welcome. Sources for every figure are on the references slide.